

File No.: EXP202305036

RESOLUTION OF RIGHTS PROCEDURE

After carrying out the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: On March 23, 2023, a complaint was filed with this Agency by Ms. A.A.A. (hereinafter, the complainant) against SOADE20, S.L. (hereinafter, the respondent) for not having duly attended to her right of access to her medical history.

The complainant states that, via a burofax dated March 6, 2023, she requested access to a complete copy of her medical history, including the name of the dental prosthetist and laboratory that participated in her treatment.

Furthermore, she indicates that, on March 14, 2023, the respondent partially addressed her request, noting that the following was missing: the CT scan that the complainant underwent at another clinic and provided to the respondent, photographs taken during the treatment, and the names of the dental prosthetist and the laboratory that were involved in her treatment.

SECOND: The respondent stated in its response to the complaint transfer process carried out by this Agency that

“At no time has the patient been denied access to her complete medical history; in fact, as mentioned in the complaint, it was provided to her in digital format, and she signed for this delivery. The invoices were provided in physical form.

Our clinic has responded at all times to Ms. A.A.A.'s complaint regarding consumption (a copy of which we attach).

Our clinic has responded to the burofax sent by Ms. A.A.A., as well as to a second email sent, which we attach. (...)

The CT scan is a diagnostic test that cannot be provided to the patient. It is in radiographic paper format, and this test is necessary to present as a diagnosis of the work done, as she has undergone implant surgery. This CT scan must remain in our office, as it is mandatory for operating on a patient. She has been informed that she can collect a digital copy at the radiological center where it was performed, but the patient has not wanted to go to collect it. We cannot request a copy of it, as it must be the patient who requests it, and we also cannot provide her with a copy of ours, as it is in radiological format, which cannot be copied.

Therefore, we have proceeded to take photographs of the CT scan in radiographic format, so that Ms. A.A.A. can have it. (...)

In light of the above, this Agency, in its resolution dated June 2, 2023, considered that the right had been attended to.

THIRD: Dissatisfied with the resolution, the complainant filed a motion for reconsideration arguing that the response received is incomplete, as the detailed invoices were not provided, only the general ones; the delivery note from the laboratory; the certificate of conformity for the porcelain crowns supposedly made; and the 50 photographs of her face and mouth.

After instructing the corresponding Reconsideration Procedure, in which the respondent was given the opportunity to be heard, on October 9, 2023, the Director of this Agency upheld the motion because the respondent did not prove that it had provided the data requested by the appellant or that it had reasonably denied the delivery of such data.

FOURTH: As a result of the acceptance of the aforementioned Reconsideration Motion, the present Rights Procedure was opened, transferring the documentation to the respondent, so that within ten working days it could present the arguments it deemed appropriate, without any written submission being made.

LEGAL GROUNDS

Competence

In accordance with the powers granted to each supervisory authority by Article 58.2 of Regulation (EU) 2016/679 (General Data Protection Regulation, hereinafter GDPR), and as established in Articles 47, 48.1, and 64.1 of the LOPDGDD, the Director of the Spanish Agency for Data Protection is competent to initiate and resolve this procedure.

Furthermore, Article 63.2 of the LOPDGDD states that: "The procedures processed by the Spanish Agency for Data Protection shall be governed by the provisions of Regulation (EU) 2016/679, by this organic law, by the regulatory provisions issued in its development, and, in so far as they do not contradict them, subsidiarily by the general rules on administrative procedures."

II

Preliminary Issues

In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for Data Protection is competent to perform the functions assigned to it in its Article 57, among them, to enforce the Regulation and promote the...

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raising awareness among the controllers and processors regarding the obligations that pertain to them, as well as addressing the complaints submitted by a data subject and investigating, as appropriate, the reasons for such complaints.

Correspondingly, Article 31 of the GDPR establishes the obligation for controllers and processors to cooperate with the supervisory authority that requests it in the performance of its functions. In the event that they have appointed a data protection officer, Article 39 of the GDPR assigns this officer the function of cooperating with said authority.

The initiation of this procedure occurs due to the failure to address a request for the exercise of the rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, which states:

"When the procedure refers exclusively to the failure to address a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679 of the European Parliament and of the Council, of April 27, 2016, it shall be initiated by an admission agreement, which shall be adopted in accordance with the provisions of Article 65 of this organic law.

In this case, the deadline for resolving the procedure shall be six months from the date on which the admission agreement was notified to the complainant. After this period, the interested party may consider their complaint to be upheld."

Article 58.2 of the GDPR grants the Spanish Data Protection Agency a series of corrective powers for the purpose of rectifying any infringement of the GDPR, including "ordering the controller or processor to address the requests for the exercise of the rights of the data subject under this Regulation."

III

Rights of individuals regarding personal data protection

The rights of individuals regarding personal data protection are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. The rights of access, rectification, erasure, objection, the right to restriction of processing, and the right to data portability are contemplated.

The formal aspects related to the exercise of these rights are established in Articles 12 of the GDPR and 12 of the LOPDGDD.

Additionally, consideration is given to the provisions expressed in Recitals 59 and following of the GDPR.

In accordance with the provisions of these regulations, the data controller must establish formulas and mechanisms to facilitate the exercise of rights by the data subject, which shall be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than one month, unless it can demonstrate that it is unable to identify the

data subject, and to express its reasons in the event that it does not intend to address such requests.

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request. The burden of proof regarding compliance with the duty to respond to the request for the exercise of rights made by the affected party falls on the data controller.

The communication addressed to the interested party regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language.

IV

Right of Access

In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, "the interested party has the right to obtain from the data controller confirmation as to whether or not personal data concerning them is being processed and, if so, the right of access to the personal data." Regarding the right of access to personal data, according to the provisions of Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the data controller may request the affected party to specify the "data or processing activities to which the request refers." The right will be deemed granted if the data controller provides remote access to the data, considering the request fulfilled (although the interested party may request the information referred to in the aspects provided for in Article 15 of the GDPR).

The exercise of this right may be considered repetitive if it is exercised more than once within a period of six months, unless there is a legitimate reason for it. On the other hand, the request will be considered excessive when the affected party chooses a means other than the one offered that incurs a disproportionate cost, which must be borne by the affected party.

Like the other rights of the interested party, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them that is being processed, as well as information, in particular, about the purposes of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the data may be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the information available about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of obtaining a copy of the personal data being processed shall not adversely affect the rights and freedoms of others.

The right of access concerning medical records is specifically regulated in Article 18 of Law 41/2002, of November 14, which is the basic regulation governing Patient Autonomy and Rights and Obligations in the Field of Information and Clinical Documentation (hereinafter LAP), the literal wording of which states:

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"1. The patient has the right of access, with the reservations indicated in section 3 of this article, to the documentation of the medical record and to obtain a copy of the data contained therein. Health centers shall regulate the procedure that guarantees the observance of these rights.

2. The patient's right of access to the medical record may also be exercised by duly accredited representation.

3. The patient's right of access to the documentation of the medical record may not be exercised to the detriment of the right of third parties to the confidentiality of the data contained therein collected in the

therapeutic interest of the patient, nor to the detriment of the right of the professionals involved in its preparation, who may oppose the right of access based on the confidentiality of their subjective notes.

4. Health centers and individual practitioners shall only facilitate access to the medical records of deceased patients to persons linked to them, for family reasons or otherwise, unless the deceased expressly prohibited it and this is evidenced. In any case, access by a third party to the medical record motivated by a risk to their health shall be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of the professionals, or that harms third parties, shall not be provided."

In this regard, it is important to highlight Article 15 of the LPA, which outlines the minimum content of the medical record:

"1. The medical record shall include the information deemed essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a written record, or in the most appropriate technical support, of the information obtained in all their care processes, carried out by the health service both in the field of primary care and specialized care.

2. The main purpose of the medical record shall be to facilitate healthcare, documenting all those data that, in the medical opinion, allow for the truthful and updated knowledge of the health status.

The minimum content of the medical record shall be as follows:

- a) The documentation related to the clinical-statistical sheet.
- b) The admission authorization.
- c) The emergency report.
- d) The anamnesis and physical examination.
- e) The evolution.
- f) The medical orders.
- g) The consultation sheet.
- h) The reports of complementary examinations.
- i) The informed consent.
- j) The anesthesia report.
- k) The operating room report or delivery record.

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- l) The pathological anatomy report.
- m) The evolution and planning of nursing care.
- n) The therapeutic application of nursing.
- ñ) The vital signs chart.
- o) The clinical discharge report.

Paragraphs b), c), i), j), k), l), ñ), and o) will only be required in the completion of the clinical history when it concerns hospitalization processes or as otherwise provided.

3. The completion of the clinical history, in aspects related to direct patient care, will be the responsibility of the professionals involved.

4. The clinical history will be maintained with criteria of unity and integration, in each healthcare institution at a minimum, to facilitate the best and most timely knowledge by the healthcare professionals of the data of a specific patient in each care process."

Regarding the preservation of the clinical history, Article 17 of the LPA, in its points 1 and 5, states that:

"1. Healthcare centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original format, for the due assistance to the patient for the appropriate time in each case and, at a minimum, five years from the date of discharge from each care process (...)

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the care documentation they generate."

V

Conclusion

In the present case, from the examination of the documentation provided during the processing of the procedure, it has been established that the claimant requested access to their clinical history from the entity being claimed and that this entity has not fully attended to that access.

Therefore, it is appropriate to uphold the claim that initiated this procedure.

In view of the cited provisions and others of general application, the Director of the Spanish Agency for Data Protection RESOLVES:

FIRST: TO UPHOLD the claim made by Ms. A.A.A. considering that the provisions of Article 15 of the GDPR have been infringed and to urge SOADE20, S.L. with NIF ***NIF.1, to send the claimant a certification within ten working days following the notification of this resolution, addressing the requested right or denying it with justification indicating the reasons why the request cannot be attended to, in accordance with the provisions established in the body of this resolution. The actions taken as a result of this Resolution must be communicated to this Agency within the same period.

Non-compliance with this resolution could result in the commission of an infringement of Article 83.6 of the GDPR, classified as very serious for the purposes of prescription in Article 72.1.m) of the LOPDGDD, which will be sanctioned in accordance with Article 58.2 of the GDPR.

SECOND: NOTIFY this resolution to Ms. A.A.A. and to SOADE20, S.L.

In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Director of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution or directly file an administrative contentious appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and in section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Contentious-Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.

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Mar España Martí

Director of the Spanish Agency for Data Protection

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